MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 6 OF 28

MFT 706

Clinical Practice and Techniques

16 teaching weeks · 10 practicals · 0 worked cases · 1 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701, 702

> “This is the skills course.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 706 — Clinical Practice and Techniques

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 16 teaching weeks · Prerequisite: MFT 701, 702

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	**Clinical Practice and Techniques**	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the skills course. You will leave able to structure a session, ask six kinds of question deliberately, conduct an enactment, reframe without dismissing, assign a task that gets done, and end therapy well. Most of the hours are practice, not lecture.

How to use this book

Technique is a set of **deliberate choices**, not a personality. Each week introduces one family of skills, gives you the language, shows a transcript, and then requires you to perform it. Read the transcript aloud before the lab: your mouth needs to know the sentences before your mind can choose them under pressure.

1. Course description

The gap between knowing about family therapy and doing it is bridged by rehearsal. This course covers the micro-skills (joining, questioning, reflecting, summarising), the meso-skills (structuring a session, managing conflict, conducting an enactment), and the macro-skills (planning treatment, assigning tasks, handling impasse, terminating).

It is assessed almost entirely on **performance**, because a student who can describe an enactment and cannot conduct one is not yet a clinician.

2. Learning outcomes

#	Outcome	Assessed
LO1	Structure a session with a clear opening, middle and close	Observed session
LO2	Use six categories of question deliberately and appropriately	Observed session, exam
LO3	Conduct an enactment and manage its escalation	Observed session
LO4	Reframe, normalise and positively connote without dismissing distress	Skills test
LO5	Design and review between-session tasks	Task portfolio
LO6	Manage conflict, silence and resistance in the room	Observed session
LO7	Plan and conduct a planned ending	Viva, log

3. Assessment

Component	Weight	Due
Skills triad log (weekly)	20%	Fortnightly
Observed session 1 (20 min, mid-semester)	20%	Week 8
Task portfolio (5 designed tasks with rationale)	20%	Week 12

Component	Weight	Due
Observed session 2 (40 min, with feedback)	40%	Week 16

4. The sixteen-week plan

Week	Skill family	Practical
1	The architecture of a session	Structure a 50-minute session
2	Joining and the first five minutes	Opening rounds, three styles
3	Listening: paraphrase, reflect, summarise	Reflection-only interview
4	Questioning I: linear, circular, strategic	Circular questioning drill
5	Questioning II: reflexive, future-oriented, scaling	Solution-focused drill
6	Enactment: making the family talk to each other	Enactment lab
7	Reframing, normalising, positive connotation	Reframe tennis
8	**Observed session 1**	Assessment and feedback
9	Working with conflict and escalation	De-escalation drill
10	Silence, tears, and the therapist's discomfort	Tolerance-of-silence lab
11	Structuring: who sits where, who speaks when, subgroups	Subgroup session design
12	Between-session tasks and homework	Task portfolio workshop
13	Impasses, non-attendance, and drop-out	Difficult-session rehearsal
14	Working with children and adolescents in family sessions	Live supervision
15	Endings: planned, unplanned, and therapeutic	Termination role-play
16	**Observed session 2**	Full assessment

Week 1 — The architecture of a session

A 50-minute family session has five movements. Beginners skip the first and the last, which is why their sessions feel formless.

Movement	Minutes	Purpose
1. Opening / re-joining	3–5	Reconnect, state the frame, ask what has changed
2. Agenda	2–3	Whose concern is most urgent this week?
3. Work	25–30	The core intervention: enactment, questioning, restructuring
4. Consolidation	5–7	Summarise, check, agree the task
5. Close	3–5	Confirm next appointment, name one thing to watch

The "what has changed?" opener. Never start where you left off. Start with the family's own observation: "Before we begin — what has been different this week, even slightly?" This trains the family to notice change and gives you data about what is already working.

> **PRACTICAL**
> Write the five movements on a card. Use it in every triad this semester. By Week 8 you should not need it.

Week 2 — Joining

Three joining styles (choose deliberately):

1. **Tracking** — follow the family's language and pace. "Tell me about 'stubborn' — what does it look like in your house?"
2. **Accommodation** — adopt the family's structure for the moment. With a hierarchical family, address the father first; with a chaotic family, bring structure immediately.
3. **Personal joining** — offer a small, true piece of yourself. "I grew up in a house where nobody raised their voice either. It took me years to learn to argue."

What joining is not. It is not agreement, flattery, or becoming the family's friend. You can join and disagree: "I like the way you two can laugh at this. I am going to say something you may not like."

Three failures of joining.

Failure	Symptom	Repair
Over-joining	You are now part of the family's pattern	Name it: "I think I just took your side. Let me step back."
Under-joining	Family answers in monosyllables	Slow down; ask about something concrete and safe
Premature confrontation	Family defensive from minute five	Return to their account for ten minutes before challenging

> **PRACTICAL**
> Three rounds of five-minute openings, each with a different joining style. The "family" rates: did I feel understood? Did I feel patronised? Did I want to continue?

Week 3 — Listening

Paraphrase — return the content. **Reflect** — return the feeling. **Summarise** — return the pattern across people and time.

Client: "He comes home, eats, and goes to the room. I sit in the parlour with the children and I feel like a widow."

- Paraphrase: "So most evenings he is in the room and you are with the children."
- Reflect: "You feel alone in your own marriage."
- Summarise (later, across people): "So there are two versions of the evening: yours is that he leaves; his is that he arrives already exhausted and afraid of saying the wrong thing. Both of you are alone in the same house."

The 2:1 rule for beginners. Two reflections for every question. It slows the interview, produces richer material, and prevents the interrogation pattern families experience in first sessions.

> **PRACTICAL**
> Ten-minute interview in which you may not ask a single question. Only paraphrase, reflect and summarise. Note how uncomfortable it is, and what the client does when you stop asking.

Week 4 — Questioning I

Six categories, each with a purpose. Learn the label so you can choose under pressure.

Type	Purpose	Example
Linear	Gather facts	"When did the arguing start?"
Circular	Reveal difference and relationship	"Who notices first when your mother goes quiet?"
Strategic	Introduce a direction	"What would happen if you stopped reminding him?"
Reflexive	Invite a new perspective	"If your daughter could hear this conversation, what would she say you both believe?"
Future-oriented	Open possibility	"Suppose this is better in six months — what would be different?"
Scaling	Make change measurable	"On 0–10, how close are you as a couple? What would one point look like?"

The circular question is the field's signature skill. Build it from three elements: a difference (who/what/how much), a relationship (between whom), and a comparison (more than, less than, compared with).

- Weak: "Is your husband supportive?"
- Better: "Who in the family would say your husband is most supportive — and who would disagree?"
- Best: "If your mother were watching the two of you last night, what would she say was different between how he spoke to you and how he speaks to your sister?"

> **PRACTICAL**
> Circular questioning drill: 10 minutes, all questions must be circular. Observer counts and classifies. Target: 80% genuinely circular.

Week 5 — Questioning II

Reflexive questions invite the family to observe itself. They are the safest way to raise something difficult, because the family supplies the content:

- "What do you think your son has learned about marriage from watching the two of you?"
- "If I asked your daughter who is in charge in this house, what would she say?"
- "What is the most useful thing that could happen in this session — and the most uncomfortable?"

Future questions create hope without denying the present:

- "Suppose we meet in a year and this is behind you. What would you want to be able to say you did?"
- "What would your father have wanted for this family?"

The question to avoid: "Why?" in relation to behaviour. "Why did you shout at her?" invites defence or invention. "What happened just before you shouted?" invites sequence.



Figure 5.1 — Solution-focused moves: strengths, miracle, scaling, exceptions, task

PRACTICAL

Convert ten "why" questions into "what/how/who" questions. Compare the answers you get in role-play.

Week 6 — Enactment

Definition. Enactment is the deliberate direction of family members to speak **to each other** in the session, while the therapist moves to the edge of the action.

Why it matters. Most families come to therapy to talk about each other to a third party. Enactment converts description into live interaction, where you can observe and shape it. It is the single most powerful structural technique.

The four-step protocol.

1. **Block the detour:** "Don't tell me — tell her."
2. **Specify the content:** "Tell her what you needed on Saturday, in one sentence."
3. **Direct the process:** "Look at him, not at me." "Say it again, slower."
4. **Intensify or interrupt:** "You just smiled. Say it again with the smile." Or: "Stop. What just happened?"

Transcript (structural enactment with a couple).

THERAPIST:

You have both told me about Saturday. Now I want you to tell each other. Funmi, tell Femi what you needed on Saturday — one sentence, and look at him. **Funmi:** (to therapist) I needed him to — **Therapist:** To him. **Funmi:** I needed you to call and say you would be late. **Femi:** I did call — **Therapist:** Femi, answer her, not me. **Femi:** (pause) I did call. At four. You didn't pick up. **Funmi:** I was with your mother. **Therapist:** Stop there. Funmi — what happened inside you when he said that?

Hazards. Enactment can escalate. Have a stop signal, keep your voice calm and slow, and interrupt before contempt. With coercive control, do not enact conflict; safety first (MFT 704).

PRACTICAL

Enactment lab. Triads with a scripted couple. The therapist may only use the four steps. Observer records: did the therapist stay out of the content? Did the family speak to each other for more than 90 seconds?

Week 7 — Reframing, normalising, positive connotation

Reframing changes the frame, not the facts. It is the most-used intervention in the field and the most easily abused.

Original frame	Reframe	Risk
"He is controlling"	"He is frightened of losing you and tries to hold on"	Can excuse control
"She is manipulative"	"She has learned that tears are the only way to be heard"	Can remove accountability
"You are lazy"	"You are exhausted and have stopped asking for help"	Can be heard as excusing
"Our son is defiant"	"Your son is trying to protect his mother"	Can burden the child further

Three tests before you reframe.

1. **Is it true enough?** A reframe that the family rejects as false damages alliance.
2. **Does it open or close action?** "He is frightened" opens action; "he is a monster" closes it.
3. **Does it remove accountability?** Reframes should increase the family's sense of agency, not reduce it.

Normalising — placing behaviour in a developmental or statistical context: "Arguing about money is the most common argument in the first five years of marriage. It is not a sign that you are failing."

Positive connotation (Milan) — describing all behaviour as serving the family system: "Your arguments may be the family's way of ensuring that nothing important is decided without both of you being fully present." Use sparingly; it can sound like sarcasm.

> **PRACTICAL**
> "Reframe tennis": one student states a complaint, the partner must return three successive reframes, and the first student says which one she would accept and why.

Week 8 — Observed session 1

Twenty minutes with a role-played family. Assessed against five criteria: structure (does the session have a shape?), joining (does each person feel heard?), questioning (at least three circular), one enactment, and a summary with a task.

Feedback is given live by the observing group using the structured sheet, then by the lecturer.

Week 9 — Working with conflict and escalation

The therapist's job during conflict is regulation, not resolution. Five moves:

1. **Slow the pace.** "Stop. Say that again, more slowly."
2. **Name the process.** "I notice that every time she raises her voice, you leave the room. Can we stay with it for two minutes?"

3. **Structural interruption.** "I am going to ask you both to be quiet for one minute and let me speak to your son."
4. **Separate content from process.** "The topic is money. The pattern is that you both start explaining before the other finishes."
5. **Time-out with a return.** "Let's take two minutes. We are not finished."

Escalation signs to interrupt early: raised volume, contempt (eye-rolling, mockery), one person standing, threats, children becoming still or leaving, your own heart rate rising.

> **DE-ESCALATION DRILL**
> Pairs escalate a scripted argument on cue. The therapist must interrupt before the third escalation. Repeat until the interruption happens at the right moment, not the safe one.

Week 10 — Silence, tears, and the therapist's discomfort

Silence is a technique. There are four kinds:

Silence	Meaning	Response
Processing	Someone is thinking	Wait. Count to fifteen.
Emotional	Feeling has arrived	Stay; soften your face; do not rescue
Resistance	Something is not being said	Name it gently: "Something just shifted. What happened?"
Your own	You are lost	Say so: "Let me think for a moment about what I am hearing."

Tears. Do not hand a tissue immediately; it tells the person to stop. Wait three seconds, then offer. Ask: "Is this a sadness that has been waiting a long time?"

Your discomfort is data. If you want to fill the silence, ask what you are avoiding. If you feel irritated with one family member, ask what they represent to you. This is the beginning of using the self as an instrument (developed in MFT 720 and 722).

Week 11 — Structuring the room and the system

Physical structure. Seat families so that coalition patterns are visible and can be interrupted. Put the parents together and the children together; sit between people who never speak; never sit in a line that recreates the hierarchy you are trying to change.

Who attends. Deliberately choose the combination: whole family, parents only, siblings, one parent and one child, individual. State the rationale: "Next week I would like to see the two of you alone, so you can talk as a couple without the children carrying it."

Subgroup sessions. A powerful and under-used technique: see the parents alone to plan a united message, then bring the children in and have the parents deliver it together. The intervention is the difference between the two sessions.

> **PRACTICAL**
> Design a three-session sequence using different attendance combinations for the Adeyemi family (MFT 701). Justify each combination.

Week 12 — Between-session tasks

A task is a test of the system, not homework. It should be small, specific, behavioural, and designed to produce information.

The four-part task formula.

1. **What exactly** ("For the next seven days, at dinner…")
2. **Who does what** ("…each of you names one thing that went well in your day…")
3. **What to notice** ("…and notice what happens to the atmosphere by Thursday.")
4. **What to bring back** ("…we will look at what you noticed next week.")

Five useful task types.

Type	Purpose	Example
Observation	Gather data	"Notice and write down every time the argument stops before it starts."
Ritual	Interrupt a sequence	"One night this week, the parents go out for one hour. The children are told in advance."
Behavioural exchange	Build positives	"Three small caring acts each per day, unannounced."
Communication	Practise a skill	"Two 15-minute speaker–listener conversations, timer on."
Paradoxical (rare, supervised)	Change the symptom's function	"On Tuesday, have the argument on purpose for ten minutes at 8 p.m."

Why tasks fail. Too big, too vague, agreed under pressure in the last two minutes of the session, or the family does not believe the therapist believes they will do it. Write the task down and give them the paper.

> **TASK PORTFOLIO (20%)**
> Five tasks, each with: the presenting problem, the task, the rationale, the predicted failure mode, and the review question you would ask.

Week 13 — Impasses, non-attendance and drop-out

When a session goes flat. Three likely causes: the family is protecting a secret; the therapist has joined too closely to one side; or the work has reached material the family did not consent to. Name the process: "I notice we have been very polite for twenty minutes. What are we all avoiding?"

When someone does not attend. Never pretend it did not happen. "Your father chose not to come today. Let's think about what that means." Explore whether attendance is a form of communication.

When the family drops out. Follow up once, by phone, briefly and without pressure; record the reason if given; and use it as clinical data. In family therapy, drop-out is the most common negative outcome, and it is usually visible early — the first two sessions predict the rest.

Impasses with the therapist. If you are bored, angry, or over-involved, that is information about the system. Take it to supervision before you act on it.

Week 14 — Working with children and adolescents in family sessions

Practical adjustments.

- Shorter sessions for younger children (30–40 minutes), or split sessions.
- Explain the frame to the child directly, not only to the parents: "In this room we talk, and sometimes we draw."
- Give the child a legitimate role: "You are the expert on what happens at school."
- Protect the child from being made the messenger: "That is a question for your mother to answer, not you."
- Never ask a child to choose sides in front of the parents.

The adolescent rule. Speak to the adolescent as the primary person for at least five minutes. Ask their version first. Parents' accounts of adolescents are systematically more negative than adolescents' own reports and than teachers' reports.

> **LIVE SUPERVISION**
> One student conducts a family session with a trained role-play family while the group observes; the supervisor may call in with one suggestion. Debrief with the family's experience.

Week 15 — Endings

Planned ending. Agree it in advance; review what changed and what did not; name the risk of relapse; write a one-page relapse plan with the family; leave the door open.

The four questions of a good ending.

1. What is different now, in your own words?
2. What did each of you do that made the difference?
3. What is most likely to make it come back?
4. What will you do when you notice the first sign?

Unplanned endings. Client-initiated endings are common; acknowledge and, where possible, send one closing message. Therapist-initiated endings (illness, placement change, moving) require a proper handover session — an abrupt ending can replicate an abandonment for the family.

Do not adopt. A warm ending can slip into a friendship. Say what the relationship was: "I have enjoyed working with you. This has been a professional relationship, and it ends here — that is what makes it safe."

> **PRACTICAL**
> Termination role-play: a family that has improved and is anxious about stopping. The therapist must review, name the risk, write the plan and close, in 15 minutes.

Week 16 — Observed session 2

Forty minutes, full assessment, using the criteria below and a self-assessment submitted in advance.

Criterion	Weight
Structure and pacing	20%
Joining and alliance with each member	20%
Questioning (range and timing)	20%
One deliberate intervention well executed	25%
Summary, task and closing	15%

Glossary

Accommodation — adapting one's style to a family's organisation in order to join it. **Circular question** — a question about difference or relationship that reveals pattern. **Enactment** — directing family members to interact with each other in session. **Intensity** — raising the emotional or interactional stakes to produce change. **Positive connotation** — framing all behaviour as serving the family system. **Reflexive question** — a question inviting the family to observe itself. **Reframe** — relabelling behaviour within a new, usually relational, frame. **Subgroup session** — seeing part of the family deliberately, then using the difference. **Unbalancing** — deliberately supporting one member to shift the system's equilibrium.

Self-assessment

Multiple choice

1. The correct response to "she is manipulative" as a first formulation is: (a) agree (b) challenge the parent (c) gather specific behavioural sequences (d) diagnose

2. "Who notices first when your mother goes quiet?" is: (a) linear (b) circular (c) strategic (d) closed

3. The main purpose of enactment is to: (a) gather history (b) observe and shape live interaction (c) calm the family (d) demonstrate expertise

4. A reframe should: (a) excuse the behaviour (b) open new action without removing accountability (c) always be positive (d) replace diagnosis

5. Two reflections for every question is recommended because it: (a) fills time (b) reduces interrogation and enriches material (c) prevents emotion (d) shortens sessions

6. A task is most likely to fail when it is: (a) written down (b) small and specific (c) agreed in the last two minutes (d) behavioural

7. Therapeutic silence after an emotional statement should usually be: (a) filled quickly (b) held for 10–15 seconds (c) ended by offering a tissue (d) followed by a question

8. In a family with coercive control, enactment of conflict is: (a) recommended (b) contraindicated (c) neutral (d) required

9. Asking "why did you shout?" is discouraged because it invites: (a) facts (b) defence or invention (c) circular information (d) emotion

10. A good ending includes: (a) a new diagnosis (b) a relapse plan (c) a friendship (d) a referral to another therapist

Answer key

1 c · 2 b · 3 b · 4 b · 5 b · 6 c · 7 b · 8 b · 9 b · 10 b

Short answer

1. Write one example of each: linear, circular, reflexive and scaling question, on the same topic.
2. Give the four steps of enactment with the exact words you would use for step 1.
3. What are the three tests of a good reframe?
4. Name four reasons between-session tasks fail.
5. How would you structure a session for a family with a 7-year-old and a 15-year-old?

Essay (choose one, 1,800 words)

1. "Technique is the servant of the alliance." Discuss with reference to your own observed sessions.
2. Analyse a transcript of your own session and identify three moments where a different choice was available. Justify each alternative with theory.
3. How should a family therapist adapt technique for a family in which the father is structurally absent?

Further reading

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Tomm, K. (1988). Interventive interviewing: Part III. Family Process, 27(1), 1–15.
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- Pilling, S., & Fornells-Ambrojo, M. (2015). Supervision and Consultation. UCL Competence Frameworks.

Next: MFT 707 — Assessment and Diagnosis in Family Therapy